



Redacted

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
IPAA Discrepancy		
Primary Malignancy History		
Dual/Synchronous Primary		
Cases (clerk)		
Reviewer Initials	Date Reviewed: 1/21/13	

***** MODIFIED REPORT - REVIEW ADDENDUM SECTION *****

DIAGNOSIS

(A) LEFT BREAST, SKIN-SPARING TOTAL MASTECTOMY:

TWO FOCI OF INVASIVE LOBULAR CARCINOMA, NOTTINGHAM HISTOLOGIC GRADE 1 (WELL-DIFFERENTIATED). (SEE COMMENT 1)

AT 6 O'CLOCK INVASIVE CARCINOMA MEASURES 1.5 X 0.4 CM IN CONTIGUOUS SECTIONS/SLICES.

INVASIVE CARCINOMA IS PRESENT 0.6 CM FROM CLOSEST INFERIOR MARGIN.

FOCAL DUCTAL CARCINOMA IN SITU (DCIS), LOW GRADE, SOLID PATTERN.

AT 2 O'CLOCK INVASIVE CARCINOMA MEASURES 3.5 X 3.0 CM IN CONTIGUOUS SECTIONS/SLICES.

MARGINS ARE WIDELY FREE, PRESENT AT LEAST 1.0 CM FROM CLOSEST SUPERIOR MARGIN.

FOCAL DUCTAL CARCINOMA IN SITU (DCIS), LOW GRADE, SOLID AND CRIBRIFORM PATTERNS.

Associated fibroadenoma.

Atypical lobular hyperplasia, atypical ductal hyperplasia.

Secretory changes.

Reactive changes consistent with previous biopsy site.

Dense fibrosis consistent with capsule.

Microcalcifications present in association with invasive and in situ carcinoma, and with benign breast tissue.

Nipple and skin, no tumor present.

Skeletal muscle, no tumor present.

Intact breast implant, gross examination only.

(B) LEFT AXILLARY SENTINEL LYMPH NODE #1, EXCISIONAL BIOPSY:

MICROMETASTASIS PRESENT IN ONE OF ONE LYMPH NODE. (SEE COMMENT 2)

TUMOR CELLS ARE HIGHLIGHTED BY CYTOKERATIN IMMUNOSTAIN AND ARE PRESENT WITHIN AN AREA MEASURING 0.75 X 0.1 MM IN LARGEST SINGLE SLIDE DIMENSION

(C) LEFT AXILLARY SENTINEL LYMPH NODE #2, EXCISIONAL BIOPSY:

SINGLE CYTOKERATIN POSITIVE ISOLATED TUMOR CELL PRESENT IN ONE OF ONE LYMPH NODE, MEASURES 0.01 MM IN GREATEST SINGLE SLIDE DIMENSION. (SEE COMMENT 3)

(D) LEFT AXILLARY SENTINEL LYMPH NODE #3, EXCISIONAL BIOPSY:

Fibroadipose tissue, no lymphoid tissue identified.

Immunohistochemical stain for cytokeratin is negative.

ICD-O-3
Carcinoma, lobular infiltrating
Site: @ Breast NOS
8520/3
9/20/13
250.9

COMMENT

1) Immunohistochemical stains for biologic markers are pending.

2) In specimen B, the frozen sections are reviewed but the tumor cells are not evident. In H&E stained permanent sections, the largest group of tumor cells can be identified but the tumor cells are bland and well differentiated. The tumor cells are best seen on immunostained slides and are present in four areas in the lymph node sections. The majority of the tumor cells are clustered within a 0.75 x 0.1 mm area. In addition, isolated tumor cells, singly or in pair, are present in three other regions of the lymph node sections.

3) In specimen C, tumor cells are not evident in the frozen and permanent H&E sections.

GROSS DESCRIPTION

(A) LEFT BREAST, SHORT SUPERIOR, LONG LATERAL - A 16.0 x 15.0 x 7.0 cm skin sparing total mastectomy specimen that is oriented by the surgeon with a short stitch designating superior and a long stitch designating lateral. The specimen is partly surfaced by a 4.5 x 3.5 cm ovoid portion of pale tan wrinkled skin. Located on the surface of the skin is an everted nipple surrounded by a rim of areola that measure 1.2 and 3.5 cm in diameter, respectively. The specimen is serially sectioned from medial to lateral into eleven slices. The nipple is located in slice #5. In slices #2 through #10 there is a 12.0 x 12.0 x 2.7 cm breast implant and cavity lined by a pink-white smooth, glistening, fibrous capsular tissue. The specimen contains an intact brand breast implant filled with a translucent viscous material. Identifying information is as follows:

The breast implant measures 12.0 x 12.0 x 2.7 cm and is for gross only. Radiographs are taken of the specimen and display at the 6 o'clock position, within slices #5 and #6, a 1.5 x 1.0 x 0.5 cm firm, gray-white mass that grossly comes to within 0.6 cm from the anterior inferior margin, 3.5 cm from the deep margin, 3.5 cm from the nipple and abuts the implant capsule. There is a metal biopsy clip located in this mass in slice #5. At the 2 o'clock position, in slices #8 through #10, there is a 3.5 x 3.0 x 1.3 cm firm gray-white mass that grossly comes to within 3.0 cm from the deep margin, 1.0 cm from the anterior superior margin, 2.5 cm from the anterior inferior margin, and abuts the implant capsule. There is a metal biopsy clip located in slice #9 in this mass.

Remaining cut surfaces are 60% yellow glistening adipose tissue and 40% gray-white fibrous tissue. Some tissue has been given to tumor bank for research purposes.

INK CODE: Blue - superior; orange - inferior; black - deep.

SECTION CODE: A1, A2, nipple from slice #5; A3, A4, 6 o'clock mass to inferior margin from slice #5; A5, slice #4, capsule and inferior margin, medial to 6 o'clock mass; A6, mass slice #6; A7, slice #7, lateral to 6 o'clock mass; A8, slice #7, lateral to 2 o'clock mass; A9, 2 o'clock mass, slice #8; A10, 2 o'clock mass, slice #9; A11, 2 o'clock mass in area of clip, slice #9; A12, 2 o'clock mass with superior margin, slice #9; A13, deep margin from slice #9; A14, 2 o'clock mass, slice #10; A15, representative sections from slice #11 (lateral margin), perpendicularly sectioned and adjacent to 2 o'clock mass; A16, upper inner quadrant from slice #3; A17, upper inner quadrant from slice #4.

*IOA/DX: MARGINS ARE FREE GROSSLY/RADIOLOGIC.

(B) LEFT AXILLARY SENTINEL LYMPH NODE #1, BLUE, IN VIVO 65, EX VIVO 117 - A 1.1 x 0.7 x 0.4 cm lymph node.

SECTION CODE: B, one lymph node serially sectioned for frozen section diagnosis.

*FS/DX: NO TUMOR PRESENT.

(C) LEFT AXILLARY SENTINEL LYMPH NODE #2, BLUE, IN VIVO 65, EX VIVO 27 - A 1.1 x 0.7 x 0.4 cm lymph node.

SECTION CODE: C, one possible lymph node serially sectioned for frozen section diagnosis.

*FS/DX: NO TUMOR PRESENT.

(D) LEFT AXILLARY SENTINEL LYMPH NODE #3, BLUE, IN VIVO 65, EX VIVO 24 - A 2.0 x 1.5 x 0.5 cm portion of fibroadipose tissue. The tissue is dissected; however, a definite lymph node is not grossly identified. Cut surfaces are composed of unremarkable fibroadipose tissue.

SECTION CODE: D, entire specimen.

*IOA/DX: NO GROSSLY IDENTIFIED LYMPH NODE. DEFER TO PERMANENT.

CLINICAL HISTORY

Left breast cancer.

SNOMED CODES

T-04050, M-Y2003

"Some tests reported here may have been developed and performance characteristics determined by specifically cleared or approved by the U.S. Food and Drug Administration."

These tests have not been

Start of ADDENDUM

ADDENDUM

COMMENT

This report is issued to give Immunohistochemistry results. Immunohistochemical staining is performed in our lab on a representative paraffin-embedded section of INVASIVE LOBULAR CARCINOMA, left breast, block A4, 6 o'clock.

MARKER	Clone/Vendor	RESULTS	STAINING	SCORE
Estrogen Receptor	6F11-	Positive	95 %	Strong
Progesterone Receptor	PgR1294()	Positive	95 %	Strong
Ki-67	MIB1 ()	Low	5 %	N/A
HER2 overexpression	AB8 ()	Negative	N/A 1+	

This report is issued to give Immunohistochemistry results. Immunohistochemical staining is performed in our lab on a representative paraffin-embedded section of INVASIVE LOBULAR CARCINOMA, left breast, block A11, 2 o'clock.

MARKER	Clone/Vendor	RESULTS	STAINING	SCORE
Estrogen Receptor	6F11-	Positive	>95 %	Strong
Progesterone Receptor	PgR1294()	Positive	>95 %	Strong
Ki-67	MIB1 ()	Low	5 %	N/A
HER2 overexpression	AB8 ()	Negative	N/A 1+	

COMMENT

Due to the above HER2 immunohistochemical staining result, gene copy level (HER2: CEP17 signal ratio) will be evaluated by FISH and a separate report will be issued.

FOOTNOTE

Breast specimens used for determining prognostic / predictive markers are fixed in formalin for 6-48 hours.

For ER & PR: Positive 10 - 100%
Low Positive 1-9%

For Ki-67 Low Positive <17%
Moderate Positive 17-35%
High Positive >35%

For Her 2: Positive cases are those with uniform, intense and complete membrane staining in greater than 30% of invasive tumor cells (Score 3+). Negative cases are defined as those with no staining (Score 0) or weak, incomplete membrane staining in any proportion of cells (Score 1+). Equivocal or indeterminate cases are those with strong staining in less than or equal to 30% of cells, or complete membrane staining that is either non-uniform or weak in intensity, but with obvious circumferential distribution in at least 10% of tumor cells (Score +2).

Reference Wolff AC, Hammond EH, Swartz JN et al. American Society of Clinical Oncology/College of American Pathologists Guideline Recommendations for Human Epidermal Growth Factor Receptor 2 Testing in Breast Cancer. Arch Pathol Lab Med. 2007;131:18-43.

Start of ADDENDUM #2

ADDENDUM

For FISH results, please see

-----END OF REPORT-----